

Clinical Diagnosis & History:
//o female with left breast IDC

Specimens Submitted:

- 1: SP: Level three left axillary lymph nodes
- 2: SP: Level two left axillary lymph nodes
- 3: SP: Lateral left breast skin
- 4: SP: Right upper abdominal skin tag
- 5: SP: Left breast and level 1 axillary lymph nodes

UUID: 6C202D3E-5678-4863-82A2-9EE7AC385089
TCGA-AO-A0JB-01A-PR

Redacted



DIAGNOSIS:

- 1) SOFT TISSUE, LEVEL III LEFT AXILLA; EXCISION:
 - BENIGN FIBROADIPOSE TISSUE.
 - NO LYMPH NODE IDENTIFIED.
- 2) LYMPH NODES, LEVEL II LEFT AXILLA; EXCISION:
 - THREE BENIGN LYMPH NODES (0/3).
- 3) SOFT TISSUE, LATERAL LEFT BREAST; EXCISION:
 - ONE BENIGN LYMPH NODE (0/1).
 - BENIGN FIBROADIPOSE TISSUE.
- 4) SKIN, RIGHT UPPER ABDOMINAL TAG; EXCISION:
 - IRRITATED SEBORRHEIC KERATOSIS.
- 5) BREAST AND LEVEL I AXILLARY LYMPH NODES, LEFT; MASTECTOMY:
 - INVASIVE DUCTAL CARCINOMA, HISTOLOGIC GRADE III/III (SLIGHT OR NO TUBULE FORMATION), NUCLEAR GRADE II/III (MODERATE VARIATION IN SIZE AND SHAPE), WITH AREAS OF LOBULAR GROWTH, MEASURING 11.0 CM IN LARGEST DIMENSION GROSSLY.
 - DUCTAL CARCINOMA IN SITU (DCIS) IS ALSO IDENTIFIED, SOLID TYPE, WITH MODERATE HIGH NUCLEAR GRADE AND EXTENSIVE NECROSIS.
 - LOBULAR INVOLVEMENT BY DCIS IS PRESENT.
 - THE DCIS CONSTITUTES $\leq 25\%$ OF THE TOTAL TUMOR MASS, AND IS PRESENT ADMIXED WITH AND AWAY FROM THE INVASIVE COMPONENT.
 - THE INVASIVE CARCINOMA IS LOCATED IN THE UPPER OUTER QUADRANT, UPPER INNER QUADRANT AND LOWER OUTER QUADRANT.
 - THE DCIS IS LOCATED IN THE UPPER OUTER QUADRANT, UPPER INNER QUADRANT AND LOWER OUTER QUADRANT.

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1CD-0-3

carcinoma, infiltrating duct, NOS 8500/3
Site: breast, NOS C50.9 lw 10/24/11

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	lw	lw 10/24/11

- NO INVOLVEMENT OF THE NIPPLE BY EITHER IN SITU OR INVASIVE CARCINOMA IS IDENTIFIED.
- CALCIFICATIONS ARE PRESENT IN THE IN SITU AND INVASIVE CARCINOMA, AND IN BENIGN BREAST PARENCHYMA.
- EXTENSIVE VASCULAR INVASION IS PRESENT.
- NO INVOLVEMENT OF THE SURGICAL MARGINS BY EITHER INVASIVE OR IN SITU CARCINOMA IS IDENTIFIED.
- NO SKIN INVOLVEMENT BY CARCINOMA IS IDENTIFIED.
- THE ATTACHED SKELETAL MUSCLE IS UNINVOLVED BY CARCINOMA.
- THE NON-NEOPLASTIC BREAST TISSUE SHOWS BIOPSY SITE CHANGES AND PROLIFERATIVE FIBROCYSTIC CHANGES.
- THE LYMPH NODE STATUS IS AS FOLLOWS (EXPRESSED AS THE NUMBER OF POSITIVE LYMPH NODES IN RELATION TO THE TOTAL NUMBER OF LYMPH NODES EXAMINED): LEVEL I: 1/10.
- THERE IS EXTRANODAL EXTENSION OF CARCINOMA, >2 MM.

IMMUNOHISTOCHEMICAL STAINS WERE PERFORMED ON FORMALIN-FIXED TISSUE WITH THE FOLLOWING RESULTS FOR INVASIVE CARCINOMA (BLOCK 5T5):

ESTROGEN RECEPTOR (6F11, STRONG INTENSITY	90% NUCLEAR STAINING WITH
PROGESTERONE RECEPTOR (1E2; MODERATE INTENSITY	60% NUCLEAR STAINING WITH
HER2 (HERCEPTEST; DAKO): INTENSITY OF 0)	NEGATIVE (STAINING

2

CONTROLS ARE SATISFACTORY.

COMMENT: HERCEPTESTM (DAKO) IS AN FDA-APPROVED METHOD FOR ASSESSMENT OF HER2 PROTEIN OVEREXPRESSION IN BREAST CANCER TISSUE ROUTINELY PROCESSED FOR HISTOLOGICAL EVALUATION. THE HER2 TEST RESULTS ARE REPORTED IN ACCORDANCE WITH THE ASCO/CAP GUIDELINE RECOMMENDATIONS FOR HER2 TESTING IN BREAST CANCER (J CLIN ONCOL 2007; 25(1):1-28).

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Special Studies:

Result	Special Stain	Comment
	ER-C	
	PR-C	
	HER2-C	
	IMM RECUT	
	NEG CONT	
	NEG-HER2	

Gross Description:

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MD

D.

1.) The specimen is received in formalin, labeled "level 3 left axillary lymph nodes" and consists of two irregularly shaped fragments of yellow lobulated adipose tissue measuring 1.0 x 1.0 x 0.2 cm in aggregate. No lymph nodes are identified grossly. The specimen is entirely submitted.

Summary of sections:
U-undesignated

2.) The specimen is received in formalin, labeled "level 2 left axillary lymph nodes" and consists of one irregularly shaped fragment of yellow lobulated adipose tissue measuring 3.5 x 2.5 x 0.3 cm. Three possible lymph nodes are identified, ranging from 0.3 cm to 0.5 cm in greatest dimension. The lymph nodes are entirely submitted.

Summary of sections:
U -- undesignated

4.D.

3.) The specimen is received in formalin, labeled "lateral left breast skin" and consists of one irregularly shaped fragment of yellow to tan fibroadipose tissue measuring 10.3 x 2.9 x 1.4 cm. No skin is identified grossly. Serial sectioning reveals a grossly unremarkable fibroadipose tissue with a single possible lymph node measuring 0.4 cm in greatest dimension. Representative sections are submitted.

Summary of sections:
U-undesignated

..D.

4.) The specimen is received in formalin, labeled "right upper abdominal skin tag" and consists of one polypoid shaped fragment of brown skin measuring 0.3 x 0.2 x 0.2 cm. The specimen is entirely submitted.

Summary of sections:
U-undesignated

..D.

5.) The specimen is received fresh, labeled "left breast and level 1 axillary lymph nodes, stitch marks level 1 axillary lymph nodes" and consists of a breast with attached axillary tail. The breast measures 22.2 x 16.0 x 3.5 cm with overlying skin ellipse measuring 16.6 x 7.9 cm. Situated eccentrically on the skin surface is an everted nipple measuring 1.2 x 1.1 x 0.3 cm and areola measuring 4.9 x 4.1 cm. The skin shows no scars. A suture demarcates the axillary tail which measures 9.8 x 5.5 x 1.5 cm. The posterior surface of the breast is inked black and the specimen is serially sectioned to reveal an irregularly shaped white-tan firm mass

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measuring 11.0 x 6.5 x 4.8 cm, involving upper outer, lower outer and upper inner quadrants, located 0.6 cm away from the deep resection margin. The remaining breast tissue shows predominantly yellow lobulated adipose tissue admixed with scant white-tan fibrous soft tissue with no other grossly identifiable lesions. The axillary tissue is dissected to reveal multiple lymph nodes ranging in size from 0.5 cm to 2.5 cm. The specimen is submitted for lymph node dissection. Representative sections of the mastectomy specimen and all identified axillary lymph nodes are submitted. Tissue submitted for TPS.

Summary of sections:

N - nipple
 NB - nipple base
 S - skin
 D - deep margin
 T - tumor
 UIQ - upper inner quadrant
 LIQ - lower inner quadrant
 UOQ - upper outer quadrant
 LOQ - lower outer quadrant
 LNS -- lymph nodes

4

Summary of Sections:

Part 1: SP: Level three left axillary lymph nodes

Block	Sect.	Site	PCs
1		U	1

Part 2: SP: Level two left axillary lymph nodes

Block	Sect.	Site	PCs
1		U	1

Part 3: SP: Lateral left breast skin (sr)

Block	Sect.	Site	PCs
1		U	1

Part 4: SP: Right upper abdominal skin tag

Block	Sect.	Site	PCs
1		U	1

Part 5: SP: Left breast and level 1 axillary lymph nodes

Block	Sect.	Site	PCs
1		D	1
2		LIQ	2
6		LN	11
2		LOQ	2

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1	N	1
1	NE	1
1	S	1
5	T	5
2	UIQ	2
2	UOQ	2

S

** End of Report **